CHAPTER 04
Understanding Anxiety and Anxiety Disorders

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Editorial status: Evidence-informed educational draft. Not a diagnosis, a treatment plan, or an emergency service. Requires independent clinical review, accessibility review, and Rwanda-specific referral verification before publication.

This chapter teaches the difference between common worry and anxiety disorders, describes their symptoms and types, explains treatment options, and offers low-risk skills for managing everyday anxiety. It is designed for adult readers; child- and adolescent-specific adaptations must be reviewed separately.

Learning outcomes

- Explain anxiety and why it can be protective in some situations.
- Recognize emotional, thinking, physical and behavioral signs of anxiety without self-diagnosing.
- Describe major anxiety disorders and distinguish a panic attack from panic disorder.
- Practice optional coping skills while understanding their limitations.
- Recognize when professional or emergency assessment is appropriate.
- Support someone experiencing anxiety with respect and without judgment.

04.1 What is anxiety?

Anxiety is an experience of apprehension, nervousness, fear or worry, often related to something that might happen. It can involve the mind and the body. Before a test, an interview, or a difficult conversation, a person may feel alert, notice a fast heartbeat, and think about possible outcomes. These reactions are not automatically signs of illness.

The body has protective systems that prepare us to respond to threats. When a possible threat is noticed, attention may narrow, muscles may tense, and breathing or heartbeat may change. These responses may be useful in genuine danger, but they can also appear when a situation is uncertain rather than dangerous.

Anxiety exists on a continuum: mild temporary worry; distressing but manageable anxiety; and persistent symptoms that cause substantial suffering or interfere with daily life. Someone can be deeply distressed even if others cannot see it. Anxiety is not a character flaw, weak faith, or lack of discipline.

Anxiety, fear, stress and worry

Term	Meaning and example
Fear	Usually a response to an immediate perceived threat; for example, hearing a threatening noise nearby.
Anxiety	Often oriented toward anticipated difficulty or danger; for example, worrying about a future presentation.
Stress	A response to demands or pressures, which may include anxiety, frustration or exhaustion.
Worry	A chain of thoughts about possible negative outcomes; worry is one part of anxiety, not the whole experience.

04.2 When does anxiety become a concern?

Everyone experiences anxiety occasionally. Anxiety may warrant professional assessment when worry or fear becomes unusually intense, difficult to manage, prolonged, or starts to restrict choices and everyday activities. It might affect attendance at school or work, relationships, sleep, caring responsibilities, or the ability to travel and access services.

A clinician considers the nature and duration of symptoms, functional impairment, the person's age and context, possible physical illnesses, medications or substances, and whether another condition better explains the experience. A single symptom, an online quiz, or an AI chatbot cannot determine a diagnosis.

> Helpful distinction: "I felt nervous before speaking to the class" describes a common experience. "For months I have avoided all presentations, missed classes, and felt overwhelmed by the possibility of embarrassment" describes difficulties worth discussing with a professional. Neither statement alone establishes a diagnosis.

04.3 How anxiety may appear

Thoughts and attention

An anxious person may imagine worst-case outcomes, overestimate the likelihood of danger, seek repeated reassurance, feel unable to tolerate uncertainty, or have difficulty concentrating. Thoughts can feel compelling without necessarily being accurate predictions.

Emotions

Fear, dread, irritability, nervousness, embarrassment, and a sense of being overwhelmed are possible. A person may also feel frustrated with themselves for becoming anxious, adding another layer of distress.

Physical sensations

Possible sensations include a racing heart, sweating, trembling, muscle tension, nausea, stomach discomfort, light-headedness, and changes in breathing or sleep. These sensations are real; they are not “imagined.” Yet similar sensations may arise from infections, heart or lung problems, thyroid conditions, medication effects, low blood sugar, and other causes.

Behaviors

People sometimes avoid feared situations, postpone decisions, check things repeatedly, leave places early, overprepare, or rely heavily on reassurance. Avoidance can reduce discomfort in the short term but may keep anxiety going by preventing a person from discovering that they can cope.

Area	Possible experience
Thought	“If I make one mistake, everyone will reject me.”
Body	Tense shoulders, unsettled stomach and rapid heartbeat before speaking.
Action	Skipping the meeting or repeatedly asking others whether everything is okay.
Impact	Missing important opportunities and feeling more anxious about the next event.

04.4 Why anxiety develops

Anxiety does not have one universal cause. It can reflect combinations of inherited vulnerability, temperament, stressful experiences, physical health, learned responses, environmental uncertainty and social conditions. Chronic financial pressure, displacement, violence, discrimination or lack of security can create real threats that cannot be solved by “thinking positively.”

Caffeine or other stimulants, substance use or withdrawal, sleep deprivation, and some medical problems may worsen symptoms. This does not mean that any single factor explains everyone’s anxiety. The person’s circumstances deserve careful attention before advice is offered.

04.5 Generalized anxiety disorder (GAD)

GAD involves excessive anxiety and worry about several areas of life, such as family, health, finances, or work, that are difficult to control. Restlessness, fatigue, tension, irritability, concentration problems, and sleep disturbance may accompany the worry. A professional considers symptom duration and impact; ordinary concern about real responsibilities is not automatically a disorder.

Example: A shop owner feels worried almost daily about many possible problems, even on relatively calm days. She struggles to sleep, continually seeks reassurance, and finds it difficult to complete business tasks. A clinician could explore GAD as one possibility while also examining actual economic stressors and physical health.

04.6 Panic attacks and panic disorder

A panic attack is a sudden surge of intense fear or discomfort that can produce chest discomfort, fast heartbeat, sweating, shaking, dizziness, breathlessness, tingling, or a sense of losing control. A panic attack can occur in more than one condition and does not by itself mean a person has panic disorder.

Panic disorder involves recurrent unexpected panic attacks together with persistent concern about additional attacks and/or changes in behavior related to the attacks. Some people begin avoiding places because they fear symptoms will return. Diagnosis requires clinical assessment.

What someone can do during an episode

- If physically safe, move to a calmer place, sit or stand comfortably, and notice stable features of your environment.
- Remind yourself that intense sensations can occur with panic, while recognizing that a new or unexplained episode may require medical assessment.
- If it feels comfortable, let breathing settle naturally; avoid forcing deep or rapid breaths.
- Contact a trusted person or healthcare professional if the experience is frightening, recurrent, or difficult to manage.

> Urgent medical warning: Do not assume chest pain, severe breathing difficulty, collapse, new weakness, or other serious unfamiliar physical symptoms are “just anxiety.” Seek urgent medical evaluation. A first or unusually severe episode may need assessment to rule out medical causes.

04.7 Social anxiety disorder

Social anxiety disorder involves marked fear or anxiety about situations where a person could be observed, evaluated or rejected. Examples include speaking in groups, meeting unfamiliar people, eating in public or answering questions. It can lead to avoidance and interfere with education, employment and relationships.

Being shy is not the same as having social anxiety disorder. A shy person may be uncomfortable initially yet still participate. The clinical concern is the degree of fear, avoidance, distress and impairment, not whether someone is naturally outgoing.

04.8 Specific phobias

A specific phobia involves strong, persistent fear or anxiety linked to a particular object or situation—for example, heights, animals, needles or flying—with avoidance or marked distress. The fear is greater than would be expected from the actual danger and causes meaningful difficulty. People should not be mocked or forced into frightening situations to “prove” they are brave.

Evidence-based treatment may involve carefully planned, gradual exposure with a qualified professional. Exposure should be collaborative and tailored; this handbook does not instruct people to undertake high-intensity exposure alone.

04.9 Agoraphobia and separation anxiety

Agoraphobia involves significant fear or avoidance of situations in which escape may feel difficult or help may be unavailable if panic-like or other incapacitating symptoms occur. These may include public transport, crowds, open or enclosed spaces, or being outside home alone. Agoraphobia can occur with or without panic disorder.

Separation anxiety disorder involves developmentally inappropriate and excessive fear or distress about separation from people to whom an individual is closely attached. It can affect children and adults. The assessment depends on age, circumstance, persistence, and impairment.

04.10 Conditions that can overlap with anxiety

People with anxiety may also experience depression, sleep problems or substance-related difficulties. Trauma-related disorders and obsessive-compulsive disorder can include fear and distress but are clinically classified separately from the anxiety-disorder grouping. Physical illnesses and medications can also mimic or worsen anxiety symptoms.

For example, a person who experiences repeated nightmares and fear after a traumatic event should not automatically be labelled as having GAD. A trained clinician considers the entire picture and selects treatment accordingly.

04.11 The anxiety–avoidance cycle

Anxiety can create a pattern in which discomfort leads to avoidance. Avoidance offers brief relief, but it may strengthen the belief that the situation is dangerous or unmanageable. Future anxiety then becomes more likely.

Stage	Example
Trigger	A student is invited to answer a question in class.
Prediction	“I will embarrass myself and everyone will laugh.”
Body response	Heartbeat increases and muscles tense.
Avoidance	The student stays absent on presentation days.
Short-term effect	Immediate relief from anxiety.
Longer-term effect	The fear remains untested, attendance drops, and the next presentation feels harder.

This cycle is not universal. Avoidance is sometimes protective when a situation is genuinely unsafe. The goal is not to pressure anyone into danger. Treatment can help distinguish realistic risks from anxiety-driven avoidance and plan manageable steps where appropriate.

04.12 Practical skill 1 — Grounding in the present

Purpose: To redirect attention when worry, racing thoughts or physical tension become distracting. Grounding may reduce distress for some people but is not a cure for anxiety disorders.

- Pause if you are in a safe location. Keep your eyes open if that feels better.
- Notice where your feet touch the floor or how your chair supports your body.
- Name three neutral things you can see and two sounds you can hear.
- Notice one small action you can take next, such as drinking water or returning to a manageable task.

Choice and safety: Stop if the practice increases distress, dizziness or feelings of disconnection. People who have experienced trauma may prefer movement, conversation, or focusing on an external object rather than attention to internal sensations. This is an original exercise informed by the general grounding principles in the WHO stress-management guide [3].

04.13 Practical skill 2 — Notice and question anxious predictions

Purpose: To distinguish a thought from a fact without dismissing genuine concerns. This simplified reflection is educational and does not replace cognitive behavioral therapy.

- Write one worry in a sentence: “If I ask a question, people will think I am foolish.”
- Describe the evidence that supports the thought and evidence that does not support it.
- Ask: “Is there another plausible explanation? What might I say to someone I care about?”
- Choose a realistic, manageable next action rather than trying to guarantee a perfect outcome.

> Example: “I am nervous about asking a question, but asking does not prove I am incapable. I can write my question first and ask at an appropriate time.” The aim is a more balanced view, not forced optimism.

04.14 Practical skill 3 — Make room for uncertainty

Trying to obtain absolute certainty about health, relationships or future events can lead to repeated checking and reassurance-seeking. It may help to notice when a worry cannot be answered immediately, identify what is within your control, and return to a meaningful task.

Example: A job applicant checks email every few minutes. They decide to check at reasonable intervals, ensure their phone can receive important messages, and spend time on another worthwhile activity. This is a behavioral experiment for everyday worry, not a substitute for care when anxiety is severe.

04.15 Practical skill 4 — Sleep, movement, and daily routines

Regular sleep opportunities, movement appropriate to health and ability, meals, and social connection may support general wellbeing. Reducing excessive caffeine intake may help some people. These approaches do not replace treatment for a diagnosed disorder.

Choose changes that are realistic in the context of your life. A person working irregular hours or experiencing housing insecurity may not be able to follow a rigid schedule. Support should respect practical constraints.

04.16 Four realistic learning scenarios

Scenario A — Examination worry

A student becomes tense before an examination and fears forgetting everything. She prepares a realistic study plan, practices a brief grounding exercise, rests as well as possible, and discusses her worry with a trusted teacher. If anxiety repeatedly prevents her from attending exams, she seeks professional advice.

Scenario B — Fear of speaking at work

An employee worries about speaking during meetings and starts turning down responsibilities. He recognizes a pattern of avoidance and asks for support. A therapist may discuss evidence-based CBT approaches if social anxiety disorder is suspected. No colleague should force him into humiliating exposure.

Scenario C — Panic-like sensations while traveling

A passenger suddenly feels dizzy, short of breath and afraid. Because the symptoms are new and intense, companions help the person access medical assessment instead of simply declaring it a panic attack. Later, clinicians can assess whether anxiety contributed.

Scenario D — Worry during financial insecurity

A caregiver worries about school costs and essential household expenses. Her worry has a real source. Alongside emotional support, practical help with budgeting, social resources, or financial advice may be more useful than being told to relax. If the worry becomes disabling, mental health care can also help.

04.17 How anxiety disorders are treated

Assessment: A qualified clinician asks about symptoms, duration, daily impact, physical health, medications or substances, other mental health difficulties, and the person’s preferences. Assessment should be respectful and confidential within applicable professional and legal limits.

Psychological therapies: Cognitive behavioral therapy (CBT) is a well-supported approach for several anxiety disorders. It helps people identify maintaining patterns of thinking and behavior and develop new ways of responding. Disorder-specific treatment may include carefully supervised exposure-based methods. Some people benefit from structured guided self-help or group-based support when appropriate.

Medication: Certain antidepressant medicines, especially selective serotonin reuptake inhibitors (SSRIs), may be offered for anxiety disorders after clinical assessment. Medication benefits, adverse effects, interactions and discontinuation symptoms should be discussed with a qualified prescriber. People should not begin, stop, or change prescribed medicine based on this educational handbook.

Shared decisions and follow-up: Treatment depends on severity, preferences, response, availability and clinical judgment. Progress can be monitored over time, and plans adapted if improvement is insufficient. Treatment often works, but response and recovery timelines vary by person.

Important: Benzodiazepines have dependence and other safety concerns and are not a routine long-term solution. NICE guidance for GAD limits their role to short-term crisis measures; recommendations vary by disorder and setting. Specific medicines and doses must be determined by clinicians [2].

04.18 Supporting someone with anxiety

- Listen calmly and allow the person to describe the experience in their own words.
- Avoid ridicule or statements such as “You are weak” or “Just stop worrying.”
- Ask what kind of support would be helpful rather than assuming.
- Offer practical assistance, such as accompanying them to a health appointment if they want company.
- Respect their boundaries and do not share private details without appropriate permission, except where urgent safety obligations require action.
- Encourage professional assessment if symptoms are persistent, impairing or worsening.

If a person is anxious because of an actual threat, prioritize practical safety rather than only emotion-management strategies. Supportive communication does not require agreeing that every feared outcome will happen.

04.19 When to seek help and when to act urgently

Situation	Suggested action
Occasional worry that remains manageable	Use ordinary coping skills, seek trusted support and monitor changes.
Persistent or worsening anxiety affecting school, work, relationships or sleep	Arrange an appointment with a qualified healthcare or mental health professional.
Repeated panic-like episodes or significant avoidance	Seek professional assessment, including consideration of physical causes.
New severe chest pain, collapse, serious breathing difficulty, new neurological symptoms	Seek urgent medical evaluation; do not assume anxiety.
Immediate risk of suicide, self-harm, or inability to stay safe	Seek emergency assistance through the nearest emergency department or verified local emergency service.

Before publishing in Rwanda, VUGA NDAKUMVA should insert only referral contacts that have been directly verified, including hours of operation and what services they provide. This draft intentionally does not invent hotline numbers or promise around-the-clock availability.

04.20 Seven-day optional learning and practice plan

Day	Practice (adjust or stop as needed)
1	Notice one situation that produces worry; describe thoughts, feelings and body sensations.
2	Try the grounding exercise at a comfortable time and note what helped or did not help.
3	Identify one anxious prediction and write a more balanced alternative.
4	Notice a reassurance or checking habit and consider a realistic boundary.
5	Choose one manageable wellbeing activity or supportive conversation.
6	Identify practical assistance or professional care that could address a real stressor.
7	Review what you learned, what remains difficult, and whether support is needed.

This plan is an educational reflection activity, not a treatment course. Someone with severe anxiety, trauma-related symptoms, or substantial impairment may need individualized professional support rather than self-directed exercises.

04.21 Reflection and knowledge assessment

- What is the difference between anxiety and an anxiety disorder?
- Name two physical symptoms that can accompany anxiety. Why should new severe symptoms not automatically be attributed to anxiety?
- How can avoiding a feared but safe situation sometimes make anxiety stronger over time?
- What is the difference between a panic attack and panic disorder?
- Name two evidence-based treatment approaches or care options for anxiety disorders.
- When would you recommend professional assessment rather than only self-help?

Suggested answers

1. Anxiety is a common experience; an anxiety disorder involves clinically significant symptoms, persistence and/or impairment. 2. For example, tremor and palpitations; other medical conditions can have similar symptoms. 3. Avoidance may provide temporary relief while maintaining fear and limiting opportunities to learn. 4. A panic attack is an episode; panic disorder is a defined pattern involving recurrent unexpected attacks and continuing consequences. 5. CBT and, when clinically appropriate, medication; guided self-help may be suitable in some cases. 6. When anxiety is persistent, worsening, distressing, or interferes with life, or when concerning physical or safety symptoms occur.

04.22 Glossary

Term	Plain-language explanation
Anxiety	Worry, apprehension or fear often related to anticipated difficulty.
Anxiety disorder	Clinically significant pattern of fear or worry and related difficulties.
Panic attack	Sudden episode of intense fear or discomfort with mental and physical symptoms.
Panic disorder	A disorder involving recurrent unexpected panic attacks and ongoing related fear or behavior change.
Avoidance	Staying away from situations or experiences; sometimes protective, sometimes anxiety-maintaining.
CBT	Cognitive behavioral therapy, a structured psychological treatment.
Exposure-based therapy	Professionally planned approach to approaching feared situations in a safe, gradual way.
Grounding	A skill for directing attention to the present moment.

04.23 Core references and editorial notes

[1] World Health Organization. Anxiety disorders. 8 September 2025. https://www.who.int/en/news-room/fact-sheets/detail/anxiety-disorders

[2] National Institute for Health and Care Excellence (NICE). Generalised anxiety disorder and panic disorder in adults: management (CG113), recommendations; original publication 2011, subsequently amended. https://www.nice.org.uk/guidance/cg113/chapter/Recommendations

[3] World Health Organization. Doing What Matters in Times of Stress: An Illustrated Guide. 2020. https://tdr.who.int/home/our-work/global-engagement/9789240003927

[4] US National Institute of Mental Health. Anxiety Disorders. https://www.nimh.nih.gov/health/topics/anxiety-disorders

[5] World Health Organization. Mental disorders. October 2026 update. https://www.who.int/news-room/fact-sheets/detail/mental-disorders

Publication review checklist

- Confirm all clinical information with a licensed mental health professional familiar with local practice.
- Review the safety wording for panic-like symptoms, self-harm, and referral.
- Verify Rwanda-specific services, available languages, costs, hours, and contact details.
- Translate into Kinyarwanda and French with qualified reviewers; validate clinical terms, not merely literal translations.
- Check readability, accessible layout, screen-reader order, and non-stigmatizing language.
- Set a named content owner, review date, version history, and a process for correction.

Key takeaway: Anxiety is common and not always harmful. When worry or fear becomes overwhelming, persistent, or disruptive, effective professional support is available. Practical self-help can help with everyday distress, but serious physical symptoms or safety concerns require prompt evaluation.